

Lofi Care landing page: design and content guideline for the creative team

Status: creative brief for delegation. Positioning: Lofi Care is a complete hospital and diagnostic centre management platform for Bangladesh, launched as one brand across three audiences on a single page. The online appointment system is the one module live and free to start today; the full platform opens through a "book a demo" conversation with diagnostic centres and hospitals. Scope: everything a designer and copywriter need to build the landing page. Audience strategy, page structure section by section with example copy, a full color system with hex codes, typography, layout and responsive rules, components, imagery, accessibility, discoverability, and a handoff checklist.

House rules apply to every word that ships: no em dashes, no AI-sounding filler, no fabricated statistics, and no price baked into the page that can go stale (see §9). Currency reference: 1 USD = 123 BDT.

The load-bearing honesty rule for this launch: **we lead with the complete platform, but the only thing shown as available today is the appointment module.** Every other module is "early access," "rolling out," or "talk to us," in honest words, never a finished-product claim and never a faked screenshot (§9, §14). The platform is real and defined as an 18-module system; most of it is still being built, so the page invites facilities into an early-access conversation rather than claiming a finished hospital system.

How to read this brief: §1 to §3 are the strategy and the look. §4 to §8 are the build. §9 to §17 are the rules, the guardrails, and the handoff. A designer can start from §4. A copywriter can start from §7.

1. The brief in one page

What this page is. The Lofi Care marketing site. One brand, one page, three doors, positioning Lofi Care as the complete platform to run a diagnostic centre or hospital in Bangladesh, with the appointment module as the free way in.

The one sentence the page must land. *Lofi Care runs a diagnostic centre or hospital on one connected system, and you can start today with online appointments, free.*

The three jobs, in priority order: 1. **Convince a diagnostic centre or hospital** that Lofi Care can run their whole facility, and get them to **book a demo**. This is the platform sale. 2. **Let an**

independent doctor start on the live module now: online appointments, free. 3. **Let a patient find and book their doctor.**

Primary actions: 1. Facility (diagnostic centre or hospital): **Book a demo.** 2. Doctor: **Start free** with the appointment module. 3. Patient: **Find your doctor.**

The narrative that ties it together. The appointment module is Module 5 of the full platform. So a doctor who starts free with appointments is already on the system a hospital runs on. Start small, grow into the platform. Land and expand, one brand.

Success metric. Facility demo requests (split diagnostic centre vs hospital) and doctor appointment trial starts. The patient path is a supporting conversion.

Non-negotiables. Mobile-first, fast on a 4G phone, every claim defensible, no fixed platform price on the page, and honest per-module status (\$9).

2. Positioning: the complete platform, and how we stand apart

Lofi Care is not a chamber app. It is the system that runs the facility: an 18-module hospital and diagnostic centre management platform covering patients, doctors and referrers, billing, samples and reports, imaging, inpatient, pharmacy, workforce, and analytics. The page has to make that breadth clear in the first screen, while being honest that appointments is what a visitor can switch on today.

The category we enter is narrower than us. The visible local players are chamber and prescription tools built for one doctor's desk. Lofi Care runs the whole institution. Say "one platform for your whole facility," not "another app for your chamber."

We win the facility conversation on four things, not on being the biggest: 1. **Built for Bangladesh.** Local workflows, local payment rails, Bangla and English, and the data stays in the country (\$7.10). Global enterprise systems are not built for a Chittagong diagnostic centre. 2. **One connected system.** Every counter, lab, and department reads the same patient record. No stitched-together tools. 3. **Diagnostics depth that maps to how these centres actually make money.** Referrer commissions and earnings, barcoded samples from collection to delivery, result verification and quality control, imaging, and fast report release. This is the sharpest wedge; lead the diagnostic-centre pitch with it (\$7.5). 4. **You can start today, free, on the one module that touches every patient:** appointments. No six-month procurement to see value.

The differentiation line for the page: the complete hospital and diagnostic-centre platform built for Bangladesh, that you can start using this week.

3. Audience strategy: three doors, one page

Three readers on a single page, each with a clear path.

- **Facility decision-maker** (diagnostic centre or hospital owner or manager): the primary target. Wants to know Lofi Care can run their operation and that it is credible. Path: the platform and diagnostics story, then **Book a demo**. The facility door serves two sub-audiences, diagnostic centres and hospitals, each with its own section (\$7.5, \$7.6).
- **Independent doctor**: the self-serve entry. Wants their chamber to run on time. Path: the appointment module, then **Start free**.
- **Patient**: wants to book without a phone call. Path: **Find your doctor**.

Layout approach: a platform-first hero that speaks to the facility, with the doctor and patient doors clearly visible right below it. The facility is the headline; the doctor and patient are unmistakable but secondary. One page, three doors.

4. Color system

A calm, clinical, credible base with one warm color that does the asking. It has to carry both a patient-friendly warmth and the institutional trust a hospital or diagnostic-centre owner needs. Verify every text-on-background pair against **WCAG AA** (4.5:1 body text, 3:1 large text and UI) before finalizing. Hex values are a starting point; keep the roles, tune to pass contrast.

4.1 Palette

Role	Name	Hex	Use
Primary brand	Trust Blue	 #0E5AA7	Logo, headings accent, links, key icons, section anchors
Primary dark	Deep Ink	 #0B2A4A	Body headings and dark text (softer than pure black)
Primary tint	Sky Wash	 #EAF3FC	Alternating section backgrounds, cards, callout panels
Action accent	Marigold	 #F5871F	Primary CTA buttons only. The one color that means "do this." Warm, high contrast on blue, not red
Positive micro	Calm Green	 #1F9D74	Small positive marks only (checkmarks, "available now" badge, success), used sparingly
Neutral text	Slate	 #4A5A6A	Secondary and supporting text
Neutral line	Mist	 #D9E2EC	Borders, dividers, input outlines, "early access" badges
Base	White / Off-white	 #FFFFFF /  #F7FAFC	Page and card backgrounds
Error only	Signal Red	 #D64545	Form errors and warnings only. Never a CTA, never decoration

4.2 Color rules

- **Marigold is a promise.** Only on the single most important button in each section. One primary CTA color per screen.
- **Never use red for a call to action.** In healthcare, red reads as blood, emergency, and alarm. Red is reserved for genuine errors.
- **Module status uses color honestly:** Calm Green for "available now" (appointments only), neutral Mist for "early access" or "on the roadmap." Never paint an unbuilt module in the available color (§9).
- **White and Sky Wash carry the page.** Saturated color is the exception, not the field.
- **Contrast is a hard requirement.** Check Marigold's label color before shipping.

5. Typography

Bilingual from the start (§12). One Latin family and one Bengali family that sit well together, both free and web-served for speed.

Use	Latin	Bengali	Notes
Headings	Poppins or Plus Jakarta Sans	Hind Siliguri	Modern, professional, credible. Weight 600 to 700
Body	Inter	Hind Siliguri or Noto Sans Bengali	High legibility at small sizes on phones. Weight 400 to 500

- **One Latin and one Bengali family, no more.**
- **Type scale (mobile first):** body 16px minimum, H3 20 to 22px, H2 26 to 30px, H1 32 to 40px, scaling up on desktop. Line height 1.5 for body.
- **Line length** 60 to 75 characters on desktop; full width with gutters on mobile.
- **Set numbers** in a slightly heavier weight so they read as facts.

6. Layout and responsive rules

- **Design the phone first.** Start at 360 to 390px wide. The market is mobile and most traffic will arrive from a shared link opened on a phone. Facility buyers will also view on desktop, so the desktop layout of the platform and diagnostics sections must look credible and substantial.
- **Header:** sticky, minimal. Logo left; a language toggle (EN / বাংলা) and one primary CTA ("Book a demo") right; quiet links to the doctor and patient sections. Few exit paths.
- **Sticky mobile CTA bar:** a bottom bar that always shows the primary action.
- **Container width:** about 1140px max on desktop, centered, 16px side gutter on mobile, no horizontal scroll.
- **Section rhythm:** alternate White and Sky Wash. Generous vertical spacing (64 to 96px desktop, 40 to 56px mobile).
- **Show the platform early,** honestly. Real product shots of the live appointment module; honest placeholders or schematics for modules still being built (\$8, \$9).

7. The page spine, section by section

For each section: its goal, what it must contain, example copy to start from, and visual notes. Copy is a starting point in Lofi Care's voice, not final.

7.1 Hero (platform-first, with the doctor and patient doors)

Goal: land the complete-platform positioning and the "book a demo" action in five seconds, while making the live appointment module and the patient path unmistakable.

- **H1 (option 1):** "The complete hospital and diagnostic centre platform, built for Bangladesh."

- **H1 (option 2):** "Run your whole facility on one platform, built for Bangladesh."
- **Subhead:** "Lofi Care runs a diagnostic centre or hospital on one connected system: patients, doctors and referrers, billing, samples and reports, pharmacy, inpatient, and analytics. Online appointments are live today and free to start. Talk to us about bringing the full platform to your facility."
- **Primary CTA:** "Book a demo" (Marigold). **Secondary CTA:** "Start free with Appointments" (outline).
- **Doors band directly below:** "Diagnostic centre or hospital? **Book a demo.**" · "Independent doctor? **Start free with appointments.**" · quieter: "Patient? **Find your doctor.**"
- **Product visual:** the live appointment module and a dashboard, shown honestly. No fabricated screenshots of unbuilt modules (\$9).

7.2 Trust bar

Goal: one honest credibility signal. No fake logos.

Content, using what is true: "Built in Bangladesh. Your patients' data stays in Bangladesh." Add a founding-cohort line once real ("Our founding diagnostic centres and chambers are onboarding now"). Never invent a number.

7.3 The problem

Goal: make both the facility owner and the doctor feel the pain the platform removes.

- **H2:** "A facility runs on too many disconnected systems."
- **Body, two layers:**
- **For the facility:** registration in one book, samples on paper slips, results retyped, reports delivered late, referrer commissions tracked in a notebook, and no single view of the day or the money.
- **For the chamber doctor:** the booking line is busy, the serial lives in one assistant's head, the waiting room keeps asking whose turn it is.
- **Visual:** calm iconography of disconnected pieces joining into one, not stock photos of stressed staff.

7.4 One platform: the 18 modules

Goal: show the full system in one view, grouped by area, with honest status per module. This section earns the "complete platform" claim.

Present a grid grouped by tier. Confirm the exact list and each module's true status with the Lofi Care team (\$17). Proposed grouping and honest status:

Tier	Modules	Status
Core	Facility, people & access control · Patient identity & registry · Doctor, referrer & earnings · Service catalogue & pricing · Appointment, queue & front desk · Visit & order desk · Billing, payments & counter · Credit accounts (corporate, insurance, health card)	Appointments: available now, free. Rest: early access
Diagnostics	Sample, collection, barcode & QR · Result entry, verification & quality control · Imaging & radiology · Report release, delivery & archive	Early access
Clinical	Clinical records, prescription & digital forms	Early access
Hospital	Inpatient: admission, ward, OT, emergency & ancillary	Early access
Supply	Pharmacy & inventory	Early access
Operations	Workforce, assets & compliance	Early access
Engagement	Communication, patient portal & service desk	Early access
Insight	Reporting, analytics & MIS	Early access

Rule for this section: only the appointment module carries "Available now." Everything else is "Early access" in neutral color, never "Available," and never a fabricated screenshot (\$9). If any other module is genuinely live for the founding cohort, move its badge with the team's confirmation.

7.5 For diagnostic centres (the sharp B2B hook)

Goal: the primary conversion for the diagnostic-centre owner, led by the pain that costs them most.

- **H2:** "Know what every referrer earns. Get reports out faster."
- **Body:** track referrer commissions automatically on every bill, tag and barcode every sample from collection to delivery, verify results with quality control before release, and hand patients their reports faster, across every branch, on one system. Corporate and credit accounts handled.
- **CTA:** "Book a demo."
- **Visual:** honest schematic of the sample-to-report flow and a referrer statement, labelled as early access where not yet live.

7.6 For hospitals

Goal: the conversion for the hospital buyer.

- **H2:** "One system for your whole hospital."
- **Body:** admissions and wards, OT and emergency, pharmacy and inventory, billing and credit accounts, clinical records, workforce, and analytics, all on one platform built for Bangladesh, with an append-only audit trail on every sensitive action.
- **CTA:** "Book a demo."

7.7 The live module: appointments

Goal: convert the doctor and prove the platform is real by letting anyone start on the live module.

- **H2:** "Start today with online appointments."
- **Benefit headlines**, each with a one-line explanation and a real product shot: "No more phone calls to book," "The waiting room knows the queue," "Fewer empty chairs," "See your day from your phone." Write each in Lofi Care's plain, benefit-first voice.
- **CTA:** "Start free."

7.8 For independent doctors

Goal: the self-serve door for the solo or multi-chamber doctor.

- **H2:** "Run your chamber, starting this week."
- **Body:** the appointment module, free to start, set up for you, with a public booking link for your signboard and Facebook page. It costs less than three patients a month when you upgrade. And it is the first module of the same platform a hospital runs on, so you grow into the rest when you are ready.
- **CTA:** "Start free."

7.9 The patient section

Goal: serve the third audience and reassure the buyer that patients get a good experience.

- **H2:** "For your patients: book in a minute, skip the phone call."
- **Body:** no account to create, a code to confirm the number, a reminder the day before, a known place in the queue.
- **CTA:** "Find your doctor" (routes to a participating doctor or a directory, \$17).

7.10 Data stays in Bangladesh, and control (trust)

Goal: the differentiator no international platform can claim, plus the control a facility demands.

- **H2:** "Your patients' data stays in Bangladesh, and every sensitive action is on the record."
- **Body:** data held on servers in Bangladesh, in line with the country's data-protection rules, plus an append-only audit trail that even the owner cannot edit. Do not overclaim certifications we do not hold (\$14).

7.11 Proof and the founding cohorts

Goal: credibility without fabrication, on both tracks.

- Two founding cohorts, framed as scarcity and co-building: **Founding Diagnostic Centres and Hospitals** (early-access facilities, founder pricing) and **Founding Doctors** (free appointments, testimonials).
- Real testimonials only, with name, role, and facility. Never state a user count we do not have.
- If proof is thin at launch, lead with the founder story and the Bangladesh-built promise, not invented metrics.

7.12 Pricing

Goal: free entry on appointments, no stale price, no fixed platform number.

- **Appointments:** "Start free with your first 500 appointments. No card." Anchor the paid tier with "It costs less than three patients a month." Do not paste a fixed table that can rot; show tiers with a dated figure or a current-pricing link (\$17).
- **The platform:** priced per facility, with founder pricing for the early cohort. No number on the page. "Book a demo for a quote scoped to your centre or hospital."

7.13 FAQ

Goal: answer the questions that stop each buyer. Accordion.

Cover: is this a full hospital system or just appointments (honest: appointments is live now, the rest is early access and rolls out with you, book a demo); is it built for diagnostic centres specifically (yes, referrer earnings, samples, results, reports); why not a cheaper chamber tool (we run the whole facility); is my data safe (stays in Bangladesh, full audit trail); how long to get started (appointments today, the platform on a timeline set with you); prove it (start free on appointments).

7.14 Final call to action

- **H2:** "One platform for your facility. Start today with appointments."
- **Primary CTA:** "Book a demo." **Secondary:** "Start free with Appointments."

7.15 Footer

Contact (a Bangladeshi phone number and a WhatsApp or Messenger link), a demo-request link, a simple privacy notice and terms, the language toggle, and the company line.

8. Component and UI kit

- **Buttons:** primary (Marigold fill, label color contrast-checked), secondary (Trust Blue outline), text link. 8 to 12px radius. Minimum tap target 44 by 44px.

- **Module cards / tier grid:** grouped by area, each module a chip or card with an honest status badge (Calm Green "Available now" for appointments, Mist "Early access" for the rest).
 - **Demo-request form:** short, with the fields a facility sale needs (name, facility, facility type diagnostic centre or hospital, role, phone or email, branches). Label every field; errors in Signal Red with text.
 - **Trial start form:** one or two fields.
 - **Accordion** for the FAQ.
 - **Sticky mobile CTA bar** with the primary action.
 - **States:** hover, focus (visible focus ring), disabled, loading for every interactive element.
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9. Imagery and iconography

- **Real Bangladeshi context:** hospitals, diagnostic centres, doctors, chambers. Not generic Western stock.
 - **Show only what is real.** Real product shots for the live appointment module. For modules being built, use honest labelled placeholders, schematics, or icons, never a fabricated screenshot passed off as a working product (§14).
 - **Icons:** one consistent line-icon set, one per module, in Trust Blue or Slate.
 - **Optimize images** (§13); a slow hero on 4G is a conversion loss.
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10. Copy, voice, and the honesty rules

- **Voice:** plain, confident, credible. Short sentences.
 - **House style, enforced: no em dashes**, ever (commas, colons, full stops). No AI-sounding filler ("delve", "leverage", "seamless", "robust", "elevate").
 - **The module-status honesty rule (load-bearing).** We position Lofi Care as a complete platform, and that is fair because it is what we are building. But no module beyond appointments may be described as "available" or shown as a finished product unless it truly is. Use "early access," "rolling out," or "talk to us." This is what lets us lead big and stay honest, and it protects trust with the facility buyers who matter most.
 - **Defensible claims only.** No no-show percentage stated as fact until we have measured our own.
 - **The price-rot rule.** No hard-coded price that can go stale; anchor appointments on "less than three patients a month" and "start free"; price the platform as "book a demo."
 - **The anchor line** appears in appointments pricing and once near the final CTA.
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11. Naming

Use consistently (§17). Recommended: **Lofi Care** is the platform and the brand; the live module is **Lofi Care Appointments**; the facility product is **the Lofi Care platform** (or "Lofi Care for Diagnostics and Hospitals"). Every module in §7.4 is named under the one Lofi Care platform. Naming the module now, while only it is live, avoids a rebrand when the rest arrives.

12. Language strategy

- **English primary, Bangla available.** The facility buyer and the younger doctor read English comfortably, and English reads as modern and credible. A real EN / বাংলা toggle, not machine translation baked in.
 - **What must be bilingual:** the patient-facing section and booking flow, the primary CTAs, and the trust and compliance lines.
 - Ship English with the patient path bilingual at minimum; full depth is a decision (§17).
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13. Accessibility and performance

- **WCAG AA across the page:** contrast (§4), visible focus states, real alt text, labelled form fields, error messaging in text and not color alone.
 - **Tap targets** at least 44px, with spacing.
 - **Performance is a conversion feature:** fast first load on a mid-range Android on 4G, compressed and lazy-loaded images, modern formats, two fonts only, no heavy above-the-fold video.
 - **Render the core message and CTA even if a large image is still loading.**
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14. What NOT to do

- **Never present a module that is not built as available,** and never show a fabricated screenshot of one (§9). First rule of this launch.
- **No fabricated stats or fake social proof.** No invented user counts, no stock testimonials, no fake client logos.
- **No no-show percentage stated as fact** until measured.
- **No fixed platform price on the page,** and no appointment price hard-coded in a way that can rot.
- **No red call-to-action buttons.**
- **No claim of HIPAA or SOC2 certification.** We align with good practice and keep data in Bangladesh; we do not certify.

- **No teal or green as the primary brand color** (it blends into the local competitor pack).
 - **No generic Western stock imagery.**
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15. Measurement

- **Track each action separately:** facility demo requests (split diagnostic centre vs hospital), doctor appointment trial starts, and patient "Find your doctor" clicks. The headline numbers are demo requests and trial starts.
 - **Instrument the funnel:** hero CTA click, form start, form complete, per audience.
 - **Analytics with a light touch,** behind an honest consent banner, defaulting to the privacy-preserving choice. No personal data in a URL.
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16. Discoverability

- **Facebook link preview is the real first channel.** Doctor and healthcare groups on Facebook are where this gets shared, so a clean Open Graph preview (a purpose-made share image, a strong title, a one-line description) matters most. Test the link pasted into a Facebook post before launch.
 - **Basic on-page SEO:** clear title and meta description, sensible headings, and Bangladesh, hospital, diagnostic centre, and appointment keywords in the natural copy.
 - **Favicon and app title** set, so a saved link looks legitimate.
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17. Open decisions and assumptions

Resolve with the Lofi Care team. None block starting on \$4 to \$9.

#	Decision	Why it matters	Suggested default
1	The real status of each of the 18 modules	§7.4 must be honest; this is the launch's integrity	Only appointments "Available now"; the rest "Early access"
2	Founder cohort sizes and offer, for facilities and doctors	Scarcity and onboarding capacity	A small facility cohort (about five) plus an open doctor cohort
3	The "Book a demo" channel: form, WhatsApp, phone, or all	The primary CTA must go somewhere real	A short demo-request form plus WhatsApp and phone
4	Naming: Lofi Care platform, Lofi Care Appointments module	Header, module grid, footer	As in §11
5	Patient "Find your doctor" destination	The patient CTA needs a real target	If no directory yet, route to a "how it works for patients" page and contact
6	How appointment pricing is shown	The price-rot rule	Anchor line plus "start free"; exact figure dated or pulled from source
7	Final palette sign-off and the accessible Marigold label pairing	Contrast is a hard requirement	Values in §4, verified to AA
8	Domain and brand lockup (logo, wordmark)	Header, footer, favicon, OG image	Confirm with the team
9	What proof is true at launch (facility and doctor testimonials)	§7.2 and §7.11 cannot invent it	Founder story and Bangladesh-built lines until real proof exists

Assumption to validate: beyond appointments, the modules are defined and being built, not yet self-serve-live, so the page presents them as early access. If any module is genuinely ready for the founding cohort, move its badge to "Available now" and give it a real product shot.

18. Handoff checklist for the creative team

- [] Confirm the real module statuses (§7.4, §17) before designing that section.
- [] Wireframe the mobile layout first, then desktop, following the §7 spine.
- [] Lock the palette (§4) and verify every text-on-background pair to WCAG AA.
- [] Set the two font families (§5) and the mobile-first type scale.
- [] Design the platform-first hero with the three doors (§7.1).
- [] Build the 18-module tier grid with honest status badges (§7.4).
- [] Build the diagnostic-centre and hospital sections with their demo CTAs (§7.5, §7.6).
- [] Build the component kit (§8): buttons, module cards, demo-request form, trial form, accordion, sticky mobile CTA.
- [] Produce real product shots for the live appointment module only; honest placeholders for the rest.
- [] Draft copy from the §7 examples; run it against §10 and §14, especially module status.
- [] Write real Bengali for the bilingual elements (§12).

- ☐ Create the Open Graph share image and test the Facebook link preview (\$16).
- ☐ Check performance on a mid-range Android on 4G (\$13).
- ☐ Resolve the \$17 open decisions with the Lofi Care team.